

Reason for Referral

Patient is being referred to Geriatrics for a ****Comprehensive Geriatric Assessment**** due to progressive functional decline, recurrent falls (3 in the past 6 months), suspected early cognitive impairment, and complex polypharmacy management. Family reports increasing concern regarding patient's ability to live independently.

Relevant Medical History

- Hypertension (dx 2001)
- Type 2 Diabetes Mellitus (dx 2008), HbA1c 7.4%
- Atrial fibrillation (dx 2019), on anticoagulation
- Osteoarthritis — bilateral knees
- Benign prostatic hyperplasia
- Mild chronic kidney disease (Stage 3a, eGFR 52)
- Cataract surgery (right eye, 2022)
- Hearing loss — bilateral, uses hearing aids inconsistently

Current Medications

1. Apixaban 5 mg PO BID
2. Metformin 500 mg PO BID
3. Lisinopril 20 mg PO daily
4. Amlodipine 5 mg PO daily
5. Metoprolol succinate 50 mg PO daily
6. Atorvastatin 40 mg PO at bedtime
7. Tamsulosin 0.4 mg PO daily
8. Acetaminophen 500 mg PRN

9. Donepezil 5 mg PO at bedtime (started 2 months ago)

10. Multivitamin daily

Recent Clinical Findings

- **BP:** 142/78 mmHg (orthostatic drop noted: 122/70 standing)

- **HR:** 68 bpm, irregular

- **Weight:** 158 lbs (loss of 9 lbs over 4 months)

- **MoCA Score:** 21/30 (administered 02/18/2025)

- **Timed Up & Go Test:** 18 seconds (elevated fall risk)

- **PHQ-9:** 11 (moderate depression)

Functional Status

- **ADLs:** Independent in bathing and dressing; requires reminders for medication

- **IADLs:** Daughter manages finances and transportation; patient no longer drives (license surrendered 01/2025)

- **Living Situation:** Lives alone in single-story home; daughter visits 3x/week

- **Falls:** 3 falls in past 6 months — one resulting in wrist contusion (no fracture)

Specific Questions for Consultant

1. Evaluate for major neurocognitive disorder vs. mild cognitive impairment

2. Review polypharmacy and recommend deprescribing where appropriate

3. Assess fall risk and recommend interventions

4. Provide guidance on capacity for independent living

5. Address depressive symptoms and recommend treatment plan

****Date Signed:**** March 14, 2025

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